

link has not been established.
Insomnia may be a symptom of depression; depression must therefore be treated.
If insomnia persists, the patient should be reassessed.
In patients with a major depressive episode:
Benzodiazepines and related drugs should not be prescribed alone, since this allows the depression to progress independently, along with persistent or enhanced risk of suicide.
Suicidal tendencies may be present, therefore the lowest possible amount of zolpidem should be supplied to these patients (prescription and supply) to avoid the possibility of intentional overdose.
Methods for Progressive Discontinuation Of Treatment
These should be clearly explained to the patient.
In addition to the need to taper the doses, patients should be warned of the risk of rebound effect, so as to minimize any insomnia which may arise because of the symptoms resulting from discontinuation, even if it is gradual.
Patients must be informed of possible discomfort during the tapering-off period.
Paediatric Population
Use of zolpidem in children is not recommended since no clinical studies have been performed in this population.

Elderly Subjects
The risk of accumulation requires dose adjustment (see section *Posology and method of administration*).
Patients with Respiratory Failure
The depressant effect of benzodiazepines and related drugs should be taken into account when prescribed to patients with respiratory failure (particularly since anxiety and restlessness may be warning signs of respiratory decompensation, requiring transfer to an intensive care unit).

INTERACTIONS
Hypnotic agents

Currently prescribed hypnotic agents are either benzodiazepines and related products (zolpidem, zopiclone) or H1 antihistamines. In addition to increased sedation when prescribed with other CNS depressants, or in the event of alcohol intake, the possible increase in the respiratory depressant effect when co-administered with morphine-like substances, other benzodiazepines, or phenobarbital should also be taken into account for benzodiazepines, especially in the elderly.
Central nervous system depressants
The fact that many drugs or substances can have additive depressant effects on the central nervous system and contribute to a decrease in alertness must be taken into account. These drugs include morphine derivatives (analgesic agents, antitussive agents and replacement therapies), neuroleptic agents, barbiturates, benzodiazepines, anxiolytic agents other than benzodiazepines (such as meprobamate), hypnotic agents, sedative antidepressants (amitriptyline, doxepin, mianserin, mirtazapine, trimipramine), sedative H1-antihistamines, centrally-acting antihypertensive agents, baclofen, thalidomide, antiepileptic drugs and anesthetics.
Inadvisable combinations
Alcohol (beverage or excipient)
Alcohol enhances the sedative effect of benzodiazepines and related drugs. Impaired alertness may make driving or using machines dangerous.
Patients should not consume alcoholic beverages or medicinal products containing alcohol.
Fluvoxamine
Co-administration of fluvoxamine may increase blood levels of zolpidem. Concomitant use is not recommended.
CYP450 inhibitors and inducers
Co-administration of ciprofloxacin may increase blood levels of zolpidem. Concomitant use is not recommended.

St. John's Wort
St. John's Wort has been shown to have a pharmacokinetic interaction with zolpidem. Mean zolpidem Cmax and AUC levels are lower (by 33.7% and 30.0%, respectively) when co-administered with St. John's Wort, compared to the levels with single dose administration. Co-administration of St. John's Wort may decrease blood levels of zolpidem. Concurrent use is not recommended.
Combination requiring precautions for use
Rifampicin
Decreased plasma concentrations and reduced efficacy of zolpidem due to increased hepatic metabolism. Clinical monitoring is required. If necessary, another hypnotic agent may be used.
Combinations to be taken into consideration
Other central nervous system depressants
Possible increase in next-day drowsiness and psychomotor impairment. Impaired alertness may make driving vehicles and using machines dangerous (see sections *Special warnings and precautions for use* and *Effects on ability to drive and use machines*).
Isolated cases of visual hallucinations have been reported in patients taking zolpidem with antidepressants including bupropion, fluoxetine, sertraline and venlafaxine.

Barbiturates, morphine derivatives:
Increased risk of respiratory depression, which may be fatal in the event of an overdose.
Buprenorphine
When buprenorphine is used as a replacement therapy: increased risk of potentially fatal respiratory depression. The benefit/risk ratio of this combination should be carefully assessed. Patients must be informed of the need to comply with prescribed doses.
Clozapine
Increased risk of collapse with respiratory and/or cardiac arrest.
Opioids
Concomitant use of opioids and sedatives, e.g. benzodiazepines and related drugs such as zolpidem, increases the risk of sedation, respiratory depression, coma and death due to the increased central nervous system depressant effect. Doses and duration of concomitant treatment must be limited
Clarithromycin, erythromycin, telithromycin
Mild increased sedative effects of zolpidem.

Ketoconazole, itraconazole, voriconazole
Mild increased sedative effects of zolpidem.
Nelfinavir, ritonavir-boosted protease inhibitors
Mild increased sedative effects of zolpidem.
When zolpidem was administered with ranitidine, no significant pharmacokinetic interactions were observed.

PREGNANCY AND LACTATION

Pregnancy
There is no evidence from extensive data collected during cohort studies that exposure to benzodiazepines during the first trimester of pregnancy results in any malformative effects. However, in some epidemiological case-control studies, an increased incidence of cleft lip and palate was observed with benzodiazepines. According to these data, the incidence of cleft lip and palate would appear to be less than 2 per 1000 in neonates exposed to benzodiazepines *in utero*, compared to an expected ratio of 1 per 1000 in the general population.
Decreased fetal movement (DMF) and fetal heart rate changes have been described with administration of high benzodiazepine doses during the second and/or third trimester of pregnancy. Benzodiazepine treatment at the end of pregnancy, even at low doses, may cause signs of exposure in the neonate, such as axial hypotonia and difficulty suckling, leading to poor weight gain. Although these signs are reversible, they may last for 1 to 3 weeks depending on the half-life of the benzodiazepine prescribed. At high doses, respiratory depression or apnea and hypothermia may occur in the neonate. Furthermore, neonatal withdrawal syndrome may occur, even if there are no signs of exposure. This is characterized in particular by overexcitability, agitation and tremor in the neonate, occurring some time after delivery. The time to onset depends on the elimination half-life of the medicinal product, and may be significant if the elimination half-life is long.
Based on these data, as a precautionary measure, use of zolpidem is not recommended during pregnancy, regardless of the trimester.
Women of child-bearing potential under zolpidem treatment should be instructed to contact their physician if they plan a pregnancy or if they are in the early stages of pregnancy, in order to reassess the need for treatment.
At term, if zolpidem treatment is absolutely necessary, high doses should be avoided and the effects described above taken into account during neonatal monitoring.
Lactation
Use of zolpidem is not recommended in breast-feeding women

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

Zolta have major influence on the ability to drive and use machines.
Patients who drive and use machines should be warned that, as with other hypnotics, there may be a possible risk of drowsiness, longer reaction time, dizziness, torpor, blurred/double vision and reduced alertness as well as impaired driving ability the morning after administration.
In order to minimize this risk, at least 8 hours of uninterrupted sleep is recommended between taking zolpidem and driving, using machinery and working at heights.
Driving ability impairment and behaviors such as falling asleep while driving have occurred in patients taking zolpidem alone at therapeutic doses.
Furthermore, co-administration of zolpidem with alcohol and other CNS depressants increases the risk of such behaviors. Patients should be warned not to drink alcohol or take other psychoactive substances during treatment with zolpidem.

UNDESIRABLE EFFECTS

Adverse effects are classified according to incidence using the following classification:
Very common (≥ 1/10); common (≥ 1/100 to < 1/10); uncommon (≥ 1/1000 to < 1/100); rare (≥ 1/10 000 to < 1/1000); very rare (< 1/10 000) not known (cannot be estimated from the available data)
These effects are related to the dose administered and the individual sensitivity of each patient.

System Organ Class	Frequency	Undesirable Effects
Immune system disorders	Not known	Angioneurotic oedema
Psychiatric disorders	Common	Hallucinations, agitation, nightmares, depression (see section Special warnings and special precautions for use)
	Uncommon	Confusional state, irritability, restlessness, aggression, somnambulism (see section Special warnings and special precautions for use), euphoric mood, complex sleep behaviours (see section Special warnings and special precautions for use)
	Rare	Libido disorder
	Very rare	Physical and psychological dependence, even at therapeutic doses, with withdrawal syndrome or rebound effect that may occur on treatment discontinuation
Nervous system disorders (see section Special warnings and special precautions for use)	Not known	Behavioral disturbances, anger, delirium (see section Special warnings and special precautions for use)
	Common	Impaired alertness or even drowsiness (particularly in the elderly), headache, vertigo, insomnia, cognitive disorders such as anterograde amnesia, which may occur at therapeutic doses, the risk increasing in proportion to dose
	Uncommon	Paresthesia, tremor, attention and speech disorders

	Rare	Impaired consciousness
	Not known	Ataxia, tension
Eye disorders	Uncommon	Diplopia, blurred vision
	Rare	Vision impairment
Respiratory, thoracic and mediastinal disorders	Very rare	Respiratory depression (see section Special warnings and special precautions for use)
Gastrointestinal disorders	Common	Diarrhea, nausea, vomiting, abdominal pain
Hepatobiliary disorders	Uncommon	Elevated liver enzymes
	Rare	Hepatocellular lesions, cholestatic or mixed liver injury (see section Posology and method of administration, Contraindications, and Special warnings and special precautions for use)
Metabolism and nutrition disorders	Uncommon	Appetite disorders
Skin and subcutaneous tissue disorders	Uncommon	Skin rash, pruritus
	Rare	Urticaria
Musculoskeletal and connective tissue disorders	Uncommon	Arthralgia, myalgia, muscle spasm, muscle hypotonia
Infections and infestations	Common	Upper and lower respiratory tract infections
General disorders and administration site conditions	Common	Asthenia
	Rare	Balance disorders or even falls

Reporting suspected adverse effects

Reporting suspected adverse effects after the drug has been approved is important. This makes it possible to continuously monitor the benefit/risk ratio of the drug. Healthcare professionals are asked to report all suspected adverse effects via the national reporting system.

OVERDOSAGE

Overdose can be life-threatening, particularly multiple overdose involving other central nervous system depressants (including alcohol). In the case of zolpidem, the prognosis has always been positive for single-agent intoxication with doses of zolpidem of up to 400 mg.
If massive amounts of zolpidem are taken alone or concomitantly with other central nervous system depressants (including alcohol), the main sign of overdose is central nervous system depression, which may range from drowsiness to coma, depending on the amount ingested.
In non-serious cases of overdose, symptoms include mental confusion and lethargy.
In more serious cases, signs include ataxia, hypotonia, hypotension, respiratory depression and, in very rare cases, death.
If oral overdose occurred less than one hour before, vomiting should be induced in conscious patients; otherwise gastric lavage must be performed with protection of the airways. After this time point, administration of activated charcoal may be useful in reducing absorption.
Specific monitoring of cardiac and respiratory functions in a specialized setting is recommended.
Administration of flumazenil may be useful for diagnosis and/or treatment of accidental or intentional overdose with benzodiazepines.
Antagonism of the effects of benzodiazepines by flumazenil may promote neurological disorders (seizures), particularly in epileptic patients.
Zolpidem is not dialyzable.

STORAGE

Store below 30°C.
Store in the original packaging in order to protect from light and moisture.

PACKAGING

Box of 2 strips (Foil Print and Foil Back Polynium Silver Dof (Al 30 µm - EAA 30 µm)) of 10 tablets packed in a plastic bag and a package insert.

ON MEDICAL PRESCRIPTION ONLY

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